

I am a philosopher of science. My dissertation is focused squarely on addiction. I discuss what addiction is, what it is not, and why those clarifications matter for psychiatry. During the course of this research, I have landed at the intersection of philosophy of psychiatry, philosophy of mind, bioethics, research ethics, and game theory. I considered moments where someone would say, *Your brain is not supposed to work like that*, and I try to make sense of it. Sometimes I agree. Much of the time, as you will come to read, I do not.

DISSERTATION WORK

Chapter 1: Dependence Theory

In this first chapter, I explain why the separation of “addiction” from “dependence” is a diagnostically meaningful distinction. The word “addiction” (formally known as “substance use disorder” or “SUD”) receives this misleading interpretation mostly from the public, but the debate in the literature is recent enough that a proper review and comprehension-check is due. What the literature is transitioning away from, and what the public often translates SUD to mean, is something like: *You are unable to stop doing X*. Usually, this “doing X” is “doing drugs” or “drinking alcohol.” Then there is a common association of the SUD with “dependence.” *She is alcohol dependent*. In my research, I explain that the association between dependence and SUD is a conflation that has caused unacceptable misunderstandings in how each term is characterized in both social and clinical spheres. To offer clarity, I show precisely how an evaluatively *negative* interpretation of the word “dependence,” one focused on subjugation and compulsion, was the culprit causing most of this confusion. So, I explain how transitioning from an evaluatively negative interpretation of dependence to a *strictly teleological one* leaves us with a more precise and accurate definition of SUD. That is, when the success of a dependency is teleologically framed, the evaluation of the goal(s) in question—as in calling them good or bad goals—become independent of the dependency’s pathologization. In other words, it is not the dependency at fault in the case of someone having bad goals, it is the goals at fault. So even the successful achievement of outrageous goals are not special cases of goal-*achievement*; they are special cases of goal-*having*. So the psychiatric issue in these cases, if there is one to be had, must have something to do with the goals the person has, not the rather mundane and predictable fact they use some tool (even if that tool is drugs) to achieve those goals. In making this shift, I show how we can both capture intuitive cases of SUD and solve puzzling cases.

Chapter 2: The Addiction Game

An example of a puzzling case we can answer under a teleological framing of dependence concerns adaptive preferences. Suppose a person claims to successfully depend on alcohol. That is, they have a goal, they use alcohol to achieve it, successfully achieve that goal, and accept all the consequences involved. As the previous chapter explains, the current model of SUD says this person does not have SUD so long as her goals remain acceptably achieved. In this chapter, I explain why that makes sense as a diagnostic standard for pathological use—rather than just

reporting the state of the literature. In cases of alleged addiction, I argue that what is often a more valid concern is the alleged patient's values. To make this point, I connect feminist philosophical literature (Khader 2011, Terlazzo 2018) on "adaptive preference" with the current addiction literature, and argue: Under the current model of addiction, adaptive preferences are not addictions.

Chapter 3: Addiction is not Adaptive Preference

What is not debated: addictions cause harm. Because addictions cause harm, addictive behavior is often considered irrational in the game-theoretic sense. Meaning: it does not make sense that one would continuously use something, like drugs or alcohol, that runs orthogonal to her preferences. It is this self-induced contradiction of preferences that philosophers and clinicians describe as "irrational." While there is opposing literature that posits a theory of rational addiction, the word "rational" here always refers to agents with forward-looking and stable preferences. Using a simple game theoretic setup, I show how even in cases involving *unstable* preferences, rational choices do not prevent addiction.

FUTURE WORK

What Makes Something Addictive? *[Partly Drafted]*

The main psychiatric reason that drugs became the defining example of addiction is known as the dopamine theory of addiction. However, since this theory's wider adoption, one behavioral addiction—namely gambling disorder—has also been accepted into the Diagnostic and Statistical Manual of Mental Disorders (DSM). Gambling's admission into the DSM shows the limiting criterion on addiction was never dopamine; it is some unacceptable cost together with the unwarranted prediction of future reward. That cost, importantly, must relate to the user's own goals, not the harm it may do to others. Addiction can of course cause harm to others, but harm to others never constitutes addiction outright. So if we stretch attributions of addiction into a verdict like "consumer culture is addictive because it is bad for society," we make a category mistake because we cross the line from assessing the cost as self-regarding to other-regarding. Addiction broadens beyond drugs, but it is bounded by patient-relativity. The downfall of society is not evidence that someone is addicted to something, even if that something causes the downfall of society. In this paper, I examine why drug addiction became the prototypical example of addiction, and why gambling disorder makes sense to include as a behavioral addiction, in order to make a case for a stronger overall demarcation of addiction from social critique.